THE VERINEX GROUP

Client Onboarding Guide

Everything your practice needs to move its billing to Verinex Group: the questions we ask, the documents we need, the process week by week, and how we finalize go-live.

PREPARED FOR

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ACCOUNT MANAGER

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VERSION

1.0 · October 2026

CONTEXT

01 Why onboarding matters

Most billing problems in the first 90 days come from setup, not from the billing work itself: a provider who is not enrolled with a payer, ERAs that never reach the billing team, or visits that never make it from the EHR to a claim. A structured onboarding removes those problems before the first claim goes out.

This guide is the reference for that onboarding. It pairs with the interactive onboarding page, where your team answers the questionnaire, sees a document checklist built from those answers, and tracks progress to go-live.

> **Goal:** clean claims from day one, no gap in cash flow during the handover, and a measured baseline so you can see what changes.

What's inside

02	**Engagement models**	The first decision: whose systems we use
03	**Onboarding questions**	Full questionnaire with branching follow-ups
04	**Process & timeline**	Nine phases, owners and deliverables
05	**Systems & integration**	How each EHR connects to our billing
06	**Required documents**	Checklist with conditions
07	**Finalize onboarding**	Go-live checklist and sign-off
08	**Issues & resolutions**	Common problems and fixes

Roles

Client: a primary contact with authority to sign, an office or IT admin who can create user accounts, and each provider for credentialing details.
Verinex: an account manager (single point of contact), an onboarding specialist, the credentialing team and the assigned billing team.

THE FIRST DECISION

02 Engagement models

Before anything else we agree where your data lives and how visits reach our billing team. Your answer shapes the rest of the questionnaire and the document list.

MODEL A

Your EHR + your billing system

Verinex billers get named logins in your software and work there. Lowest change for your staff.

MODEL B

Your EHR + Verinex billing

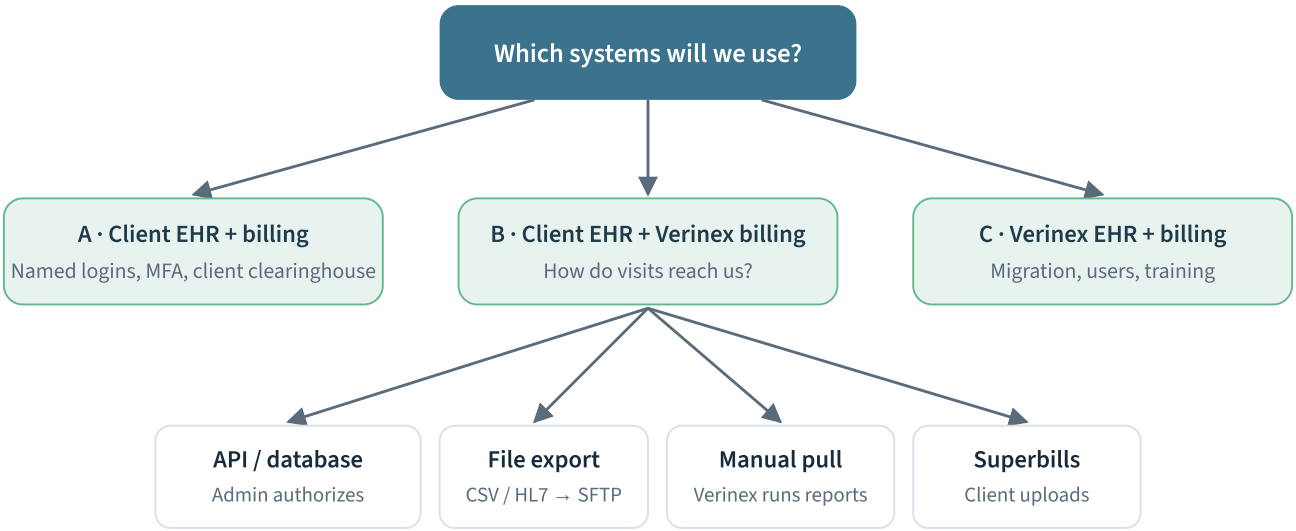
Providers keep documenting in your EHR. Visits flow into our billing platform.

MODEL C

Verinex EHR + billing

One platform for scheduling, notes and billing. We migrate data and train staff.

Decision flow



Example — Kareo / Tebra on Model B. We first ask whether you will authorize a direct connection. If yes, your system administrator generates an API key and we sync visits automatically. If your plan or policy does not allow it, our team pulls the Encounters or Appointments report on an agreed schedule, or your staff uploads superbills through the secure portal.

QUESTIONNAIRE

03 Onboarding questions

Questions marked * are required. Rows marked ↳ are follow-ups that only appear when the earlier answer shown in the last column applies. The interactive page handles this automatically.

Step 1 Practice profile

Basic facts about your practice. These drive enrollment, credentialing and claim setup.

QUESTION	OPTIONS	ASKED WHEN
Legal practice name (as on IRS letter) *	Text	Always
Primary specialty *	Behavioral health / therapy · Psychiatry · Physical / occupational therapy · Speech therapy · ABA therapy · Primary care · Internal medicine · Pediatrics · Cardiology · Orthopedics · Dermatology · Chiropractic · Laboratory · Urgent care · Other	Always
Number of rendering providers *	Number	Always
Number of service locations	Number	Always
States you practice in	Text	Always
Billing entity type *	Group practice (Type 2 NPI) · Solo provider (Type 1 NPI only) · Facility / clinic	Always
Payer mix *	Medicare · Medicaid · Medicare Advantage · Commercial · Tricare / VA · Workers' comp / auto · Self-pay	Always
Average visits per month	Number	Always
Do any providers prescribe controlled substances?	Yes · No	Always
Do you perform in-office lab testing?	Yes · No	Always

Step 2 Systems & engagement model

This is the key decision. It sets where your data lives and how visits reach our billing team.

QUESTION	OPTIONS	ASKED WHEN
Which systems will we use? *	Your EHR and your billing system · Your EHR, Verinex billing system · Verinex EHR and billing · Not sure yet	Always
Which EHR / practice-management software do you use today? *	Software list (see Section 05)	Always
↳ Software name	Text	Software = Other
↳ How will Verinex billers access your system?	Named user account for each biller · Remote desktop / VPN into your network · One shared login	Model A
↳ How is multi-factor authentication handled?	Each biller uses their own authenticator app · Codes go to a practice phone or email · MFA is not enabled	Model A
↳ Does your billing system submit claims through a built-in clearinghouse?	Yes · No, a separate clearinghouse · Not sure	Model A
↳ How should visit data reach our billing system? *	Connect directly (API / database) · Scheduled file export · Verinex pulls visit reports manually · You send superbills to us	Model B
↳ Can you authorize a connection to your EHR database or API?	Yes, I am the admin or can get approval · Our vendor has to enable it first · No, switch to manual pull	Model B + direct connection
↳ Name and email of your EHR system administrator	Text	Model B + direct connection authorized
↳ How often should visits be pulled or sent?	Daily · Twice a week · Weekly	Model B + export, manual pull or superbills
↳ Which report lists completed visits?	Text	Model B + manual pull
↳ Will billers have read-only access to clinical notes for coding review?	Yes, read-only · On request only · No	Model B
↳ What should we migrate from your current system?	Patient demographics & insurance · Future appointments · Clinical notes (as PDFs) · Open patient balances · Active authorizations	Model C
↳ How many staff need logins?	Number	Model C

QUESTION	OPTIONS	ASKED WHEN
↳ When does your current software contract end?	Date	Model C
↳ Preferred training format	Live video sessions · On-site · Recorded videos + Q&A	Model C
Who assigns CPT and ICD-10 codes? *	Providers code; Verinex reviews before submitting · Verinex certified coders code from notes · Providers code; submit as-is	Always

Step 3 Current billing status

Where things stand today, so we can set a baseline and plan any cleanup.

QUESTION	OPTIONS	ASKED WHEN
Who handles billing today? *	In-house staff · Another billing company · New practice, no billing yet	Always
↳ Current billing company name	Text	Another billing company
↳ Has notice been given to the current billing company?	Yes · Not yet	Another billing company
↳ Approximate open insurance A/R	Under $25k · $25k–$100k · $100k–$500k · Over $500k · Unknown	Existing billing
↳ Who should work claims older than our start date?	Verinex (A/R recovery project) · Previous biller runs them out · Split by date — decide on call	Existing billing
↳ Clearinghouse	Availity · Office Ally · Waystar · Optum / Change Healthcare · TriZetto · Claim.MD · Built into our software · Unknown	Existing billing
Are ERAs (835 remittances) received electronically?	Yes, from all major payers · From some payers · No, paper EOBs · Not sure	Always
Are payments received by EFT (direct deposit)?	Yes, all payers · Some payers · Paper checks	Always
Known denial issues	Text	Always

Step 4 Credentialing & enrollment

Claims only pay when every provider is enrolled with every payer. We confirm this before go-live.

QUESTION	OPTIONS	ASKED WHEN
Are all providers credentialed with the payers you bill? *	Yes, all of them · Some are pending or missing · No, we need full credentialing · Not sure	Always
↳ Should Verinex handle credentialing?	Yes, all of it · Only new providers going forward · No, we handle it	Not all providers credentialed
Is each provider's CAQH profile current and attested?	Yes · No · Not sure	Always
Any providers starting in the next 90 days?	Yes · No	Always

Step 5 Services & reporting

Choose what Verinex owns and how we keep you informed.

QUESTION	OPTIONS	ASKED WHEN
Services you want from Verinex *	Eligibility & benefits checks · Prior authorizations · Charge entry · Coding review · Claim submission · Payment posting · Denial management & appeals · A/R follow-up · Patient statements & calls · Credentialing · Monthly reporting	Always
↳ How do patients pay today?	Card at visit · Online portal · Mailed check · Payment plans	Patient statements selected
↳ Send unpaid patient balances to collections after	Never · 90 days · 120 days · 3 statements · Case by case	Patient statements selected
Primary point of contact (name, role, email) *	Text	Always
Preferred communication channel	Email · Client portal messages · Phone · Teams / Slack	Always
How often do you want performance reports?	Weekly · Every two weeks · Monthly	Always
Target go-live date	Date	Always

WEEK BY WEEK

04 Process & timeline

Typical go-live is three to five weeks after the signed agreement. Phases overlap: documents, access requests and enrollment checks run in parallel.

#	PHASE	TIMING	OWNER	WHAT HAPPENS · DELIVERABLES
1	**Discovery call**	Day 1	Verinex + Client	Walk through your practice, systems and pain points. Pick the engagement model. Engagement model chosen · Scope of services agreed · Pricing proposal
2	**Agreement & BAA**	Days 2–5	Client	Sign the services agreement and BAA. Nothing involving patient data starts before this. Signed agreement · Signed BAA · Authorized contacts named
3	**Intake & documents**	Days 3–10	Client	Complete the intake questionnaire and upload required documents through the secure portal. Questionnaire complete · Document checklist 100%
4	**System access & integration**	Days 5–15	Verinex + Client IT	Create biller logins, connect the EHR or set up the visit feed, and test a sample day of visits. • **Model A:** Your admin creates named logins with MFA for each Verinex biller. We confirm role permissions. • **Model B:** We connect your EHR to our billing system (API, export or manual pull) and reconcile one week of visits. • **Model C:** We build your Verinex EHR account, import demographics and schedule, and run staff training. Biller accounts working · Visit feed tested · Clearinghouse connected
5	**Enrollment check**	Days 5–20	Verinex	Verify every provider is active with every payer. Submit ERA, EFT and clearinghouse enrollments. Payer enrollment matrix · ERA/EFT submissions · Gaps flagged with dates
6	**Billing setup & SOPs**	Days 10–20	Verinex	Load fee schedules, payer rules and coding preferences. Agree charge-lag targets and cut-off times. Fee schedule loaded · Workflow SOP signed off · Escalation contacts

#	PHASE	TIMING	OWNER	WHAT HAPPENS · DELIVERABLES
7	**Test claims**	Days 15–25	Verinex	Submit a small batch of real claims and watch them through the clearinghouse and payer acceptance. Clean-claim rate on test batch · Rejections fixed at the source
8	**Go-live**	Day 21–30	Verinex + Client	Full daily billing starts. Handover of open A/R from the previous biller is completed. First full charge batch · A/R handover cut-off date
9	**30 / 60 / 90-day review**	Months 1–3	Verinex	Weekly check-ins for the first month, then reviews at 60 and 90 days against baseline KPIs. KPI report (days in A/R, clean-claim rate, denial rate) · Action plan

05 TECHNICAL

Systems & integration

For Model B, visits reach our billing system in one of four ways. We pick the most automated option your software and policies allow, and fall back without delaying billing.

METHOD	HOW IT WORKS	WHAT WE NEED FROM YOU
API / database connection	Visits and charges sync automatically, usually daily.	Admin authorization, vendor approval where required, API key or credentials sent through the secure portal.
Scheduled file export	Your EHR sends CSV or HL7 files to the Verinex SFTP on a schedule.	Export configured in your EHR; we supply SFTP details and file layout.
Manual report pull	Our team logs in and exports completed visits on an agreed schedule.	Named read-only login for each biller; the report name that lists completed visits.
Superbill upload	Your staff uploads encounter forms to the secure portal.	Agreed daily cut-off; sample superbill so we can map codes.

Software we commonly see

SOFTWARE	INTEGRATION	WHAT TO EXPECT
Kareo / Tebra	API available	Tebra offers an API that a system administrator enables by generating a customer key. Appointment and encounter reports can also be exported to CSV.
AdvancedMD	API via partner program	API access usually goes through the AdvancedMD partner program. Scheduled report exports are the common fallback.
athenahealth (athenaOne)	API via partner program	Integration runs through the athenahealth Marketplace. Most billing teams work directly inside athenaOne with a named user login.
eClinicalWorks	HL7 / interface	Usually needs an HL7 interface set up by the eCW interface team, often for a fee. Report exports work as a stopgap.
NextGen	HL7 / interface	HL7 or API interfaces are available on most plans. Confirm with your NextGen account manager.
Epic (Community Connect)	HL7 / interface	Data access is controlled by the host organization. Expect to work inside Epic with an assigned login.
DrChrono	API available	Has an open API. An account admin authorizes the connection.
SimplePractice	No public API	No public API. Verinex works inside the account or pulls appointment and billing exports.

SOFTWARE	INTEGRATION	WHAT TO EXPECT
TherapyNotes	No public API	No public API. Verinex works inside the account with a biller login or uses report exports.
WebPT	API via partner program	Integrations go through WebPT partners. Billing reports can be exported.
CentralReach	API via partner program	API access is available on some plans. Billing exports and in-system work are common.
Practice Fusion	No public API	Limited integration options. Plan on report exports or superbills.
Paper charts / no EHR	No public API	Visits come to Verinex as superbills or encounter forms through the secure upload portal.
Other	To be confirmed	We confirm integration options with your vendor during discovery.

Vendor capabilities and fees change. Verinex confirms the current options with your vendor during discovery.

> **Security:** every Verinex user has their own login and MFA. We never accept passwords by email or text. Access is removed the same day a team member leaves the account.

CHECKLIST

06 Required documents

Send everything through the Verinex secure portal. The interactive page shows only the items that apply to your practice.

Agreements & compliance

	DOCUMENT	WHY WE NEED IT	REQUIRED WHEN
☐	**Signed billing services agreement**	Sets scope, fees and term.	All clients
☐	**Signed Business Associate Agreement (BAA)**	HIPAA requires it before we touch any patient data.	All clients
☐	**W-9 for the billing entity**	Confirms the Tax ID that claims and payments use.	All clients
☐	**Authorized contacts form**	Who may approve write-offs, refunds and policy changes.	All clients

Practice & provider

	DOCUMENT	WHY WE NEED IT	REQUIRED WHEN
☐	**IRS EIN letter (CP 575 or 147C)**	Payers match the legal name and Tax ID exactly.	All clients
☐	**Group NPI (Type 2) confirmation**	Billing provider on group claims.	Group or facility billing
☐	**Provider roster**	Name, individual NPI, taxonomy, license number and start date for each provider.	All clients
☐	**State professional licenses**	Credentialing and payer enrollment.	All clients
☐	**DEA registration**	Required for prescribers of controlled substances.	Providers prescribe controlled substances
☐	**CLIA certificate**	Lab claims deny without the CLIA number.	In-office lab testing
☐	**Malpractice insurance face sheet**	Payers require current coverage for enrollment.	All clients
☐	**Service location list**	Address, phone and place-of-service code for each site.	More than one location

Payers & enrollment

	DOCUMENT	WHY WE NEED IT	REQUIRED WHEN
☐	**Payer contracts and fee schedules**	Lets us catch underpayments.	You bill commercial or Medicare Advantage plans
☐	**Practice charge master / fee schedule**	The billed amount for each CPT code.	All clients
☐	**Medicare PTAN and PECOS access**	Needed for Medicare claims and revalidation.	Medicare in payer mix
☐	**State Medicaid provider IDs**	Medicaid claims use a state-assigned ID.	Medicaid in payer mix
☐	**CAQH ProView logins for each provider**	Commercial credentialing pulls from CAQH.	Credentialing work or CAQH not current
☐	**Credentialing packet (CVs, board certs, diplomas)**	Payer applications ask for full history.	Verinex handles credentialing or new providers starting
☐	**Bank letter or voided check**	Needed to enroll payers in EFT.	Not all payers pay by EFT yet
☐	**Signed ERA / EFT enrollment authorizations**	Without ERAs, payments cannot be auto-posted.	ERA or EFT enrollment is incomplete
☐	**Payer portal logins list**	Claim status, eligibility and appeals.	All clients

Systems & access

	DOCUMENT	WHY WE NEED IT	REQUIRED WHEN
☐	**User accounts for Verinex billers**	One login per biller, created by your admin.	We work in or pull from your system
☐	**API / database authorization**	Admin approval or vendor form that enables the connection.	Direct connection chosen
☐	**Export schedule set up to Verinex SFTP**	Automated visit file delivery.	Scheduled file export chosen
☐	**Sample superbill / encounter form**	We map your codes and modifiers.	Visits arrive as superbills
☐	**Clearinghouse login or transfer letter**	Moves claim submission to our team or keeps yours connected.	Existing clearinghouse in place
☐	**Full data export from current software**	Source files for migration.	Moving to Verinex EHR

Historical billing data

	DOCUMENT	WHY WE NEED IT	REQUIRED WHEN
☐	**A/R aging report by payer**	Baseline for recovery and reporting.	Existing open A/R
☐	**Open claims list with claim-level detail**	So nothing falls through at handover.	Existing open A/R
☐	**Last 6 months of denials and rejections**	Find root causes before go-live.	Existing billing history
☐	**Credit balance / refund report**	Overpayments must be resolved on time.	Existing billing history
☐	**Prior biller termination and data release letter**	Authorizes the previous company to hand over data.	Switching from another billing company
☐	**Patient financial policy**	Statement cycles, payment plans and collections rules.	Verinex handles patient billing

07 READY TO BILL

Finalize onboarding

Go-live is approved when every item below is complete. Your account manager reviews the list with you on a short readiness call.

Go-live checklist

- [ ] Services agreement and BAA signed
- [ ] All required documents received
- [ ] Biller access or visit feed tested end-to-end
- [ ] Every provider active with every billed payer
- [ ] Clearinghouse enrollment approved
- [ ] ERA enrollment submitted for top payers
- [ ] Fee schedule and payer rules loaded
- [ ] Test claims accepted by payers
- [ ] A/R handover cut-off date agreed
- [ ] Reporting schedule and contacts confirmed

After go-live

Weekly check-ins for the first month, then reviews at 60 and 90 days. We report on days in A/R, clean-claim rate, first-pass resolution, denial rate and charge lag against the baseline taken during onboarding.

Sign-off

By signing, both parties confirm the go-live checklist is complete and agree the A/R handover cut-off date.

Client signatory · name & title	Verinex account manager
Signature · date	Signature · date
A/R handover cut-off date	Go-live date

TROUBLESHOOTING

08 Issues & resolutions

The problems we see most often during onboarding, who resolves them, and how to prevent them.

AREA	ISSUE	RESOLUTION	OWNER	PREVENTION
Access	**Biller logins delayed by EHR vendor**	Start the user request on day one. If the vendor charges per seat, agree who pays in the agreement. Use read-only reports in the meantime.	Client IT	Ask about seat licensing in the discovery call.
Access	**MFA codes go to one practice phone**	Move each biller to their own authenticator app. Shared codes delay work and break the audit trail.	Client admin	Set up per-user MFA before go-live.
Integration	**API connection not allowed on current EHR plan**	Switch to scheduled export or manual pull. Revisit the API when the plan renews.	Verinex	Confirm plan tier and API fees during intake.
Integration	**Visits missing between EHR and billing system**	Run a daily reconciliation: visits completed vs. charges entered. Investigate any gap within 24 hours.	Verinex	Daily charge reconciliation report.
Integration	**Duplicate charges from manual and automatic feeds**	Use one feed per provider and date range. Void duplicates before submission.	Verinex	Document a single cut-over date for each feed.
Enrollment	**Claims deny as "provider not enrolled"**	Hold claims for that provider and payer, file enrollment, and request a retro-effective date where the payer allows it.	Verinex credentialing	Enrollment matrix checked before go-live and before each new hire.
Enrollment	**Payments arrive but nothing posts**	ERA is not routed to our clearinghouse. Submit ERA enrollment; post from paper EOBs until approved.	Verinex	ERA/EFT enrollment in the first week.
Enrollment	**NPI, Tax ID or taxonomy mismatch rejections**	Match the claim to what the payer has on file (legal name, address, taxonomy). Update NPPES if it is wrong.	Verinex + Client	Verify NPPES and payer records during setup.
Transition	**Previous biller will not release data**	Send the signed data release letter. Under HIPAA the practice owns its records. Pull reports directly from the PM system where possible.	Client	Give notice and request exports before the contract ends.
Transition	**Old claims close to timely-filing limits**	Triage the aging report by filing deadline. Work the closest deadlines first and keep proof of timely filing.	Verinex	Get the open claims list in week one.

AREA	ISSUE	RESOLUTION	OWNER	PREVENTION
Transition	**Credit balances inherited from previous biller**	Review each credit, refund overpayments within payer and state deadlines, and log every decision.	Verinex + Client approver	Request the credit balance report at intake.
Operations	**High charge lag (notes not signed)**	Send providers a daily unsigned-notes list. Agree a 48-hour signing target.	Client providers	Set charge-lag targets in the SOP.
Operations	**Eligibility denials**	Verify eligibility 48–72 hours before each visit and flag inactive coverage to front desk.	Verinex	Turn on eligibility service at go-live.
Operations	**Missing prior authorizations**	Track auths by visit count and end date. Alert at 80% used or 14 days to expiry.	Verinex + Client	Load active auths during migration.
Security	**Credentials sent by email or text**	Change the exposed password right away. Re-send through the secure portal or a password manager share.	Client + Verinex	Never send logins in plain email. Use the portal.

> **Escalation:** raise any onboarding issue with your account manager. Anything that blocks claims or payments gets a same-business-day response.